

CD38: 3 molecules stopped early

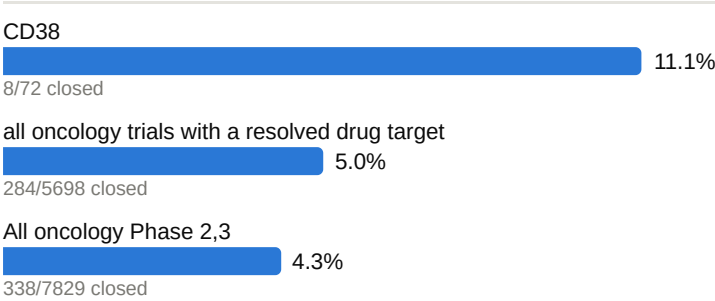
8 stopped trials across 3 molecules: Isatuximab, Daratumumab and Modakafusp Alfa. Against 5.0% for all oncology trials with a resolved drug target and 4.3% across all oncology Phase 2,3 trials in the same window. The rate counts trials that stopped early for efficacy, safety or benefit–risk reasons. Trials that ran to completion and missed their primary endpoint are counted separately below, never in the rate.

11.1%	6 / 1 / 0	34.4%	1.7%
8 of 72 closed trials (95% CI 5.7%–20.4%)	efficacy / safety / both (adds to 8)	of 209 trials have closed · 137 still open or unresolved	stopped for a biological reason within 3 years of starting (95% CI 0.6%–5.3%; 130 still at risk; 1.5% area-wide)

0 of 7

Completed and missed the primary endpoint. Of 47 completed trials in this segment, 7 posted a primary superiority analysis on ClinicalTrials.gov; 0 came back non-significant and 7 significant. Counted separately and never in the rate above. The rest posted no analysis to read, so this is a floor, not a rate.

How this compares



What is in this class: 105 distinct experimental drugs across 209 trials (antibody 205, small molecule 131, protein 49, adc 8). Most tested: DARATUMUMAB (149), DEXAMETHASONE (106), LENALIDOMIDE (55), ISATUXIMAB (50), BORTEZOMIB (45). A class groups drugs by what they act on; check that the grouping is one you would make before reading the rate as a property of the mechanism.

How much does this rest on one decision? The 8 stops came from 8 sponsor-asset programmes across 5 sponsors; the largest (Teva Branded Pharmaceutical Products R&D LLC) contributed 1; removing that programme's trials from both sides leaves 10.1%. Screened alongside 85 other segments in this area, it does not survive a 10% false-discovery correction (q=1, Benjamini-Yekutieli). A further 2 closed trials here were terminated with no cause recorded in the registry; if every one of them were biological the rate would be 13.9%. Read it as a screen worth checking against the underlying trials.

Bars show the share of closed trials stopped for biological reasons. Confidence intervals overlap where sample sizes are small — read the counts, not just the bars.

By start cohort

Trials started	Stops	Closed	Rate	95% CI
2015–2017	3	29	10.3%	3.6%–26.4%
2018–2020	5	34	14.7%	6.4%–30.1%
2021–2024	0	9	0.0%	0.0%–29.9%

Where the stops sit

Efficacy / futility: 6, Safety: 1, Benefit–risk: 1.
Sponsors with most stops: Sanofi (4), Teva Branded Pharmaceutical Products R&D LLC (1), M.D. Anderson Cancer Center (1), Fondazione Italiana Linfomi - ETS (1), Johnson & Johnson (1).

The stopped trials

Trial	Ph	Sponsor	Experimental drugs	Stopped	Type	Registry stop reason
NCT04157517	2	Teva Branded Pharmaceutical Products R&D LLC	MODAKAFUSP ALFA, PEMBROLIZUMAB	2023-12	Efficacy	Study was terminated early due to futility.
NCT03067571	2	M.D. Anderson Cancer Center	DARATUMUMAB	2022-11	Efficacy	Study terminated early because of lack of efficacy

Trial	Ph	Sponsor	Experimental drugs	Stopped	Type	Registry stop reason
NCT03860844	2	Sanofi SNY	ISATUXIMAB, DEXAMETHASONE, FLUDARABINE, CYTARABINE	2022-09	Efficacy	Study was prematurely stopped due to sponsor decision (stage 2 efficacy criteria not met); not due to safety concerns.
NCT03637764	2	Sanofi SNY	ISATUXIMAB, ATEZOLIZUMAB	2022-05	Efficacy	Study was stopped after interim analysis for 4 cohorts (HCC, SCCHN, EOC, and GBM) indicated that the results did not fulfill the preplanned interim analysis criteria to move to Phase 2 stage 2 in all 4 cohorts.
NCT04251065	2	Fondazione Italiana Linfomi - ETS	DARATUMUMAB	2021-12	Safety	The enrolment was prematurely terminated after the planned monitoring of the first 5 patients because of an unacceptable level of relevant toxicities, according to statistical stopping rules and the DSMB recommendation.
NCT03367819	2	Sanofi SNY	ISATUXIMAB, CEMPIPLIMAB	2021-03	Efficacy	The study was stopped after interim analysis indicated that overall result was not sufficient to satisfy per-protocol criteria to move forward in metastatic, castration-resistant prostate cancer (mCRPC) and non-small cell lung cancer (NSCLC) cohorts.

The 6 most recent of 8 stopped trials. Every molecule behind all 8 is named above; the complete trial list, the trials still running and the terminations with no readable cause ship with the dataset and the evidence package.

Method

Denominator: ClinicalTrials.gov interventional Phase 2,3 oncology trials started 2015–2024 that have closed (completed or terminated).
Numerator: terminated trials whose registry stop reason is classified as biological (efficacy, safety or benefit–risk) by Classification V2 — held-out precision 95.5%, recall 95.3% (n=600). Drugs are linked to ChEMBL and the NCI Thesaurus; 76% of industry oncology trials in this window carry a resolved drug target, and a trial without one cannot enter a mechanism class. Intervals are Wilson 95%.

Limits

Not a failure rate: completed trials that missed their endpoints are not in the rate. They are counted separately where the sponsor posted a primary analysis that can be read — most did not, so that count is a floor — and programmes discontinued after a completed trial do not appear. Stop reasons are sponsor-reported. This is a closed-trial proportion, not a time-to-event analysis: only 34.4% of trials in this segment have closed, and a trial that stops early enters the denominator sooner than one that runs to completion, which can inflate the rate in immature segments. Recent cohorts have fewer closed trials, so their rates are less stable. Research signals, not clinical or investment advice.

The whole cohort, trial by trial. The evidence package names every trial behind these figures — the ones still running and the ones that completed and missed their primary endpoint included — and places your molecule against each one that failed. €99, delivered immediately: clinicaltrialfailures.com/asset-check

Sources: ClinicalTrials.gov (NLM); ChEMBL (EMBL-EBI, CC BY-SA 3.0); NCI Thesaurus (NCI); RxNorm/RxClass (NLM); SEC EDGAR. Classification, linkage and rates by Clinical Trial Failures. Rebuilt weekly; this brief covers trials started 2015–2024.